جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نوسه سلامه متى بطرس
المحافظة : الجيزه **الحي / المدينة :** امبابه **العنوان :** 0
رقم البطاقة : 26109141200089 **رقم التليفون :** 01210575229 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 14-09-1961 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي الحوض [11107] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نوسه سلامه متى بطرس
National ID: 26109141200089

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي MRI of the pelvis

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تحقيق الشخصية



نورسه

سلامه متى بطرس

٨ ش يوسف الشافعي-ترعة الزمر

امبابة - الجيزه

٢٦١٠٩١٤١٢٠٠٠٨٩

LK9267050

National ID – Front
[[DOCTYPE:ID_FRONT]]



٢٦١٠٩١٤١٢٠٠٠٨٩ ٢٠٢٥/١٠
حاصله على دبلوم صنايع

انثى مسيحية متزوجة
نبيل سرور ابراهيم الديب
البطاقة سارية حتى ٢٠٣٢/١٠/٢٠



National ID – Back
[[DOCTYPE:ID_BACK]]

alfa
SCAN

ID: 5308802 Name: Nosah Salama Mata Botros
Date: 31/08/2026 Referred by: Prof. Ahmed Al Shamy

Thanks for reference

Bilateral digital soft tissue mammography & high resolution breast sonography & Elastography revealed :

Right breast:

- Scattered mixed densities of glandular condensations are noted (ACR-A).
- No speculated masses or pathological microcalcifications.
- Normal skin thickness & contour.
- Echogenic mammary parenchyma.
- Multiple right breast periareolar hyperechoic oval shaped avascular lesions are seen measuring 9-13 mm in diameter.
- No other breast solid or cystic masses.
- No architecture distortion.
- Small sized ,likely non-specific left axillary lymph nodes.

Left breast:

- Diffuse increased breast density with thickened coarsened trabeculations.
- Left LIQ obscured posterior margin isodense lesion is noted ,seen by U/S as irregular shaped infiltrative hypoechoic solid mass lesion at 8-9 O'clock ,about 5 cm from the areola ,parasternal ,measuring 63x45x32 mm in diameter with marked surrounding architectural distortion ,being vascular upon Color Doppler application.
- It is seen infiltrating the overlying skin and underlying chest wall muscles.
- Diffuse edema and thickening of the skin of the left breast with moderate central interstitial edema.
- Pathologically enlarged left axillary level I lymph node is seen measuring 32x30 mm in diameter with cortical infiltration and infiltration of the surrounding axillary fat.
- Pathologically enlarged left infraclavicular LN is seen measuring 9x5 mm .
- No pathological supraclavicular LNs.

alfa
SCAN

OPINION :

Multiple right breast lipomas **BIRADS 2.**

Left LIQ malignant looking solid mass lesion with cutaneous and chest wall infiltration ,pathological left axillary and left infraclavicular lymphadenopathies **BIRADS 5 ,for U/S guided Tru Cut biopsy.**

Kindly accept my best regards,
Dr.Sara Mahmoud,MD

Defining BIRADS classification of breast lesions according to the American College of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

GUSTAVE ROUSSY
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	نوسه سلامه متى	**Case No.**	**373024**
Sex/Age	**Female 65 Y**	**Patient No.**	**27734**
Registered Date	**07/9/2026 09:57:37 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**14/9/2026 03:18:37 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**14/9/2026 03:32:57 PM**	**Pathology No.**	**2876/26**

Pathology Report

Clinical Data: Left breast LIQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, Collected On 07/09/2026, At 02:25 pm. Cold Ischemia Time is 20 seconds.

Gross Examination:
- Four grey, white tissue cores, ranging in size between 0.5 cm & 1.2 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma with focal invasive pleomorphic lobular carcinoma component.
- **Histologic grade:** 3 (differentiation 3, pleomorphism 3, mitosis 2)
- **Ductal carcinoma in situ:** Absent.
- **Micro-calcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** 1%.

Results:
* IPT (All controls showed appropriate reactivity):
- **ER:** Strong nuclear reaction in 90% of tumor cells (score 8/8).
- **PR:** Strong nuclear reaction in 10% of tumor cells (score 5/8).
- **Her2:** Faint incomplete membrane reaction in less than 10% of tumor cells (score 0+).
- **Ki67%:** 35%.

CAP
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي الترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	نوسه سلامه متى	**Case No.**	**373024**
Sex/Age	**Female 65 Y**	**Patient No.**	**27734**
Registered Date	**07/9/2026 09:57:37 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**14/9/2026 03:18:37 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**14/9/2026 03:32:57 PM**	**Pathology No.**	**2876/26**

Conclusion:

Left Breast Mass; Tru-Cut Biopsy:

- Invasive duct carcinoma with invasive pleomorphic lobular component, Grade 3.

IPT:

- **ER:** Positive (score 8/8).
- **PR:** Positive (score 5/8).
- **Her2:** Ultra-low (score 0+).
- **Ki67%:** 35%.

***Scoring system: "ER, PR"*

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

*** Scoring system: "Her2"*

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei, M.D.

LK92670S0

٢٦١٠٩١٤١٢٠٠٠٨٩

National ID – Front
[[DOCTYPE:ID_FRONT]]

٨٩٠٠٠٢١٤٩١٢٦١ ٢٠٢٥/١٠

محمد علي دبور صقلي

الأنثى ممنوحة مؤرخة

نبيل سرور ابراهيم الليبي

١ الإطلاع سارية حتى ٢٠٢٦/١٠/٢٠

National ID – Back
[[DOCTYPE:ID_BACK]]

alfa
SCAN

ID: 5308802 Name: Nosah Salama Mata Botros
Date: 31/08/2026 Referred by: Prof. Ahmed Al Shamy

Thanks for reference

Bilateral digital soft tissue mammography & high resolution breast sonography & Elastography revealed :

Right breast:

- Scattered mixed densities of glandular condensations are noted (ACR-A).
- No speculated masses or pathological microcalcifications.
- Normal skin thickness & contour.
- Echogenic mammary parenchyma.
- Multiple right breast periareolar hyperechoic oval shaped avascular lesions are seen measuring 9-13 mm in diameter.
- No other breast solid or cystic masses.
- No architecture distortion.
- Small sized ,likely non-specific left axillary lymph nodes.

Left breast:

- Diffuse increased breast density with thickened coarsened trabeculations.
- Left LIQ obscured posterior margin isodense lesion is noted ,seen by U/S as irregular shaped infiltrative hypoechoic solid mass lesion at 8-9 O'clock ,about 5 cm from the areola ,parasternal ,measuring 63x45x32 mm in diameter with marked surrounding architectural distortion ,being vascular upon Color Doppler application.
- It is seen infiltrating the overlying skin and underlying chest wall muscles.
- Diffuse edema and thickening of the skin of the left breast with moderate central interstitial edema.
- Pathologically enlarged left axillary level I lymph node is seen measuring 32x30 mm in diameter with cortical infiltration and infiltration of the surrounding axillary fat.
- Pathologically enlarged left infraclavicular LN is seen measuring 9x5 mm .
- No pathological supraclavicular LNs.

[[DICTYPE:MAMMOGRAM]]
mammogram

alfa
SCAN

OPINION :

Multiple right breast lipomas **BIRADS 2.**
Left LIQ malignant looking solid mass lesion with cutaneous and chest wall infiltration ,pathological left axillary and left infraclavicular lymphadenopathies **BIRADS 5 ,for U/S guided Tru Cut biopsy.**

Kindly accept my best regards,
Dr.Sara Mahmoud,MD

Defining BIRADS classification of breast lesions according to the American College of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	نوسه سلامه متى	**Case No.**	**373024**
Sex/Age	**Female 65 Y**	**Patient No.**	**27734**
Registered Date	**07/9/2026 09:57:37 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**14/9/2026 03:18:37 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**14/9/2026 03:32:57 PM**	**Pathology No.**	**2876/26**

Pathology Report

Clinical Data: Left breast LIQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, Collected On 07/09/2026, At 02:25 pm. Cold Ischemia Time is 20 seconds.

Gross Examination:
- Four grey, white tissue cores, ranging in size between 0.5 cm & 1.2 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma with focal invasive pleomorphic lobular carcinoma component.
- **Histologic grade:** 3 (differentiation 3, pleomorphism 3, mitosis 2)
- **Ductal carcinoma in situ:** Absent.
- **Micro-calcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** 1%.

Results:
* IPT (All controls showed appropriate reactivity):
- **ER:** Strong nuclear reaction in 90% of tumor cells (score 8/8).
- **PR:** Strong nuclear reaction in 10% of tumor cells (score 5/8).
- **Her2:** Faint incomplete membrane reaction in less than 10% of tumor cells (score 0+).
- **Ki67%:** 35%.

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Coraiche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL EGYPTE
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	نوسه سلامه متى	**Case No.**	373024
Sex/Age	Female 65 Y	**Patient No.**	27734
Registered Date	07/9/2026 09:57:37 AM	**Referred By Dr.**	Interventional Radiology
Validation Date	14/9/2026 03:18:37 PM	**Branch**	Gustave Roussy
Reporting Date	14/9/2026 03:32:57 PM	**Pathology No.**	2876/26

Conclusion:

Left Breast Mass; Tru-Cut Biopsy:

- Invasive duct carcinoma with invasive pleomorphic lobular component, Grade 3.

IPT:

- **ER:** Positive (score 8/8).
- **PR:** Positive (score 5/8).
- **Her2:** Ultra-low (score 0+).
- **Ki67%:** 35%.

***Scoring system: "ER, PR"*

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

*** Scoring system: "Her2"*

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei, M.D.

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Nusa Salama Matti Boutros	Date : 11-10-2026 06:13
Branch : Maadi	Medical No. : 480523
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Baseline skeletal metastatic survey in a 65-year-old female with left breast carcinoma, presenting with persistent bone pain.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired approximately 3 hours after intravenous Tc-99m MDP administration.

Findings:

- Focal radiotracer hyperfixation is seen at the lower thoracic spine at T11/T12 level, with another hyperfixation along the left sacroiliac region.
- No other osseous lesions could be detected throughout the remainder of the axial and appendicular skeleton.
- Mild heterogeneous background uptake is noted throughout the remaining spine, consistent with age-related degenerative change.
- Osteoarthritic changes are noted at different bilateral weight-bearing joints as well as both shoulders.
- Normal symmetrical renal excretion with physiological bladder activity.

Impression:

- Indeterminate focal osseous uptake at the lower thoracic spine and the left sacroiliac region, for radiological correlation to exclude metastatic osteoblastic involvement in the setting of known left breast carcinoma.
- No other osseous lesions detected throughout the remainder of the axial and appendicular skeleton.
- Follow up as clinically indicated would be recommended.

Dr. Alaa Nagi, MD Nuclear Medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

THE GENERAL ORGANAIZATION
TEACHING HOSPITALS & INSTITUTES
NATIONAL HEART INSTITUTE
Tel: 3052972- 3052973
Fax: 3479893

الهيئة العامة للمستشفيات والمعاهد التعليمية
معهد القلب القومي
تليفون: 3052973-3052972
فاكس:3479893

Echo-Doppler Report

Name نوسه سلامه متى Date : Saturday, 03 October, 2026
Ref by Prof.: Dr Age : Yrs

Measurement:

L V E DD	4.9	(3.5-5.6 cm)	Left atrium	4.8	(1.9-4.0 cm)
L V E SD	3.2		Aortic root	2.6	(2.0-3.7 cm)
L V SWT	1.0	(0.7-1.1 cm)	A O, Excursion	–	(1.6-2.6 cm)
L V PWT	1.0	(0.7-1.1 cm)	Rt. Ventricle	2.3	(0.7-2.3 cm)
EF	62%		FS	33%	(25- 45%)

* Two Dimensional And M-Mode Echo Cardiographic Findings

The Mitral Valve:

Motion : Calcification: Area :
Thickness: Subvalvular : Score:

The Aortic Valve :
The pulmonary Valve :
The Tricuspid Valve :
The Left Ventricle :
The Segmental Wall Motion abnormalities
The Interventricular Septum :
The Interatrial Septum :
Masses :
Pericardium :

Doppler and Color Flow Findings:

		A/E	Max/MN. PG (Diastolic)	Max/MN. PG (Systolic)	Valve area	Regurge/Grade	Jet area
Mitral	:						
Aortic	:						
Tricuspid	:						
Pulmonic	:						

* Conclusion:

- Normal left ventricular internal dimensions with good overall global contractility .
- No resting segmental wall motion abnormalities could be detected
- Mild mitral regurgitation
- Normal aortic valve flow .
- Normal right side dimensions without evidence of pulmonary hypertension
- No intracardiac masses or thrombi

Shimaa

Signature
Dr.Mohamed El Sayed

Lab Test
[[DOCTYPE:LAB_TEST]]